

# Mystical Experience in Psychedelic-Assisted Psychotherapy

## Clinical Findings and Methodological Reflections on the MEQ30

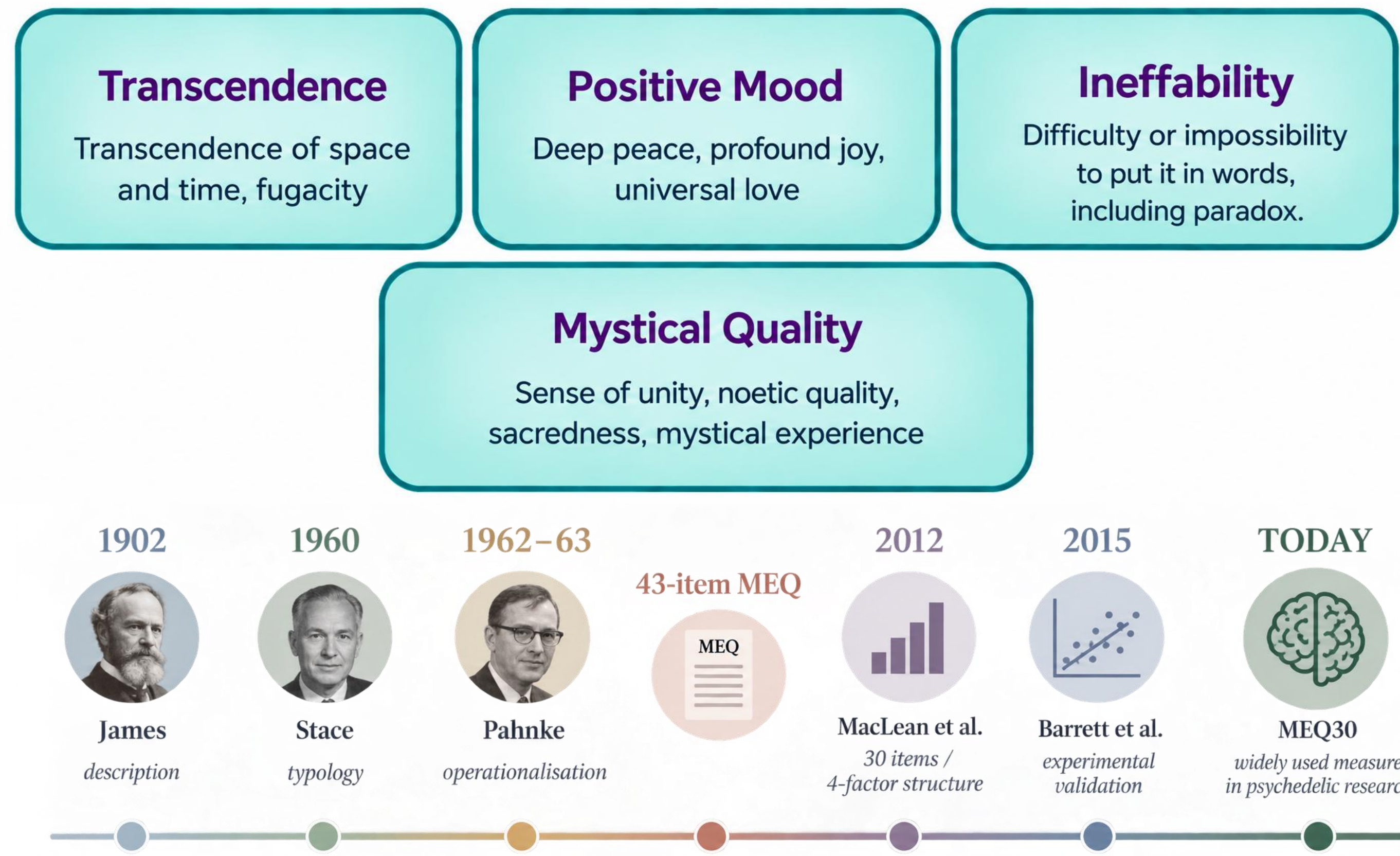


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### What is a mystical experience?

“The apprehension of an ultimate nonsensuous unity in all things.”  
— Walter T. Stace, *The Teachings of the Mystics* (1960)



### What is the MEQ30?

- A self-report measure developed to quantify the degree to which an experience displays characteristics historically associated with mystical experience.
- 30 items, rated from 0 (“none; not at all”) to 5 (“extreme”), grouped into four factors: Mystical Quality, Positive Mood, Transcendence of Time and Space, and Ineffability.
- A “complete mystical experience” is conventionally defined as reaching  $\geq 60\%$  of the maximum score on each of the four factors.
- The MEQ30 is widely used in psychedelic research to quantify mystical-type experiences and investigate their relationship with subsequent psychological and clinical outcomes.
- Higher mystical-experience scores have frequently been associated with better therapeutic outcomes, although this relationship is not consistent across studies.
- While psychometrically validated, the MEQ30 necessarily reduces complex subjective experiences to predefined dimensions, raising questions about cultural framing, individual interpretation, and what aspects of the phenomenology may remain unmeasured.

## An example from philosophy to practice

### Context and aims

Under the Swiss compassionate-use framework, the HUG psychedelic-assisted psychotherapy (PAP) programme provides LSD- or psilocybin-assisted psychotherapy to a clinical population. This retrospective study included 57 patients who completed the programme between June 2020 and March 2024.

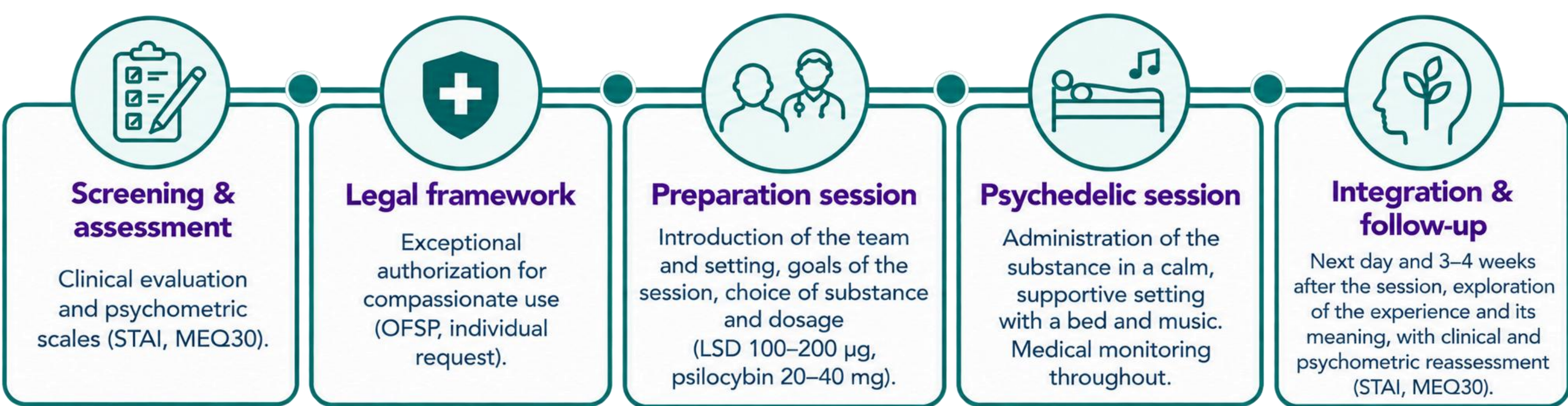
#### Central question

Is the intensity of mystical experience associated with improvement in anxiety following psychedelic-assisted psychotherapy?

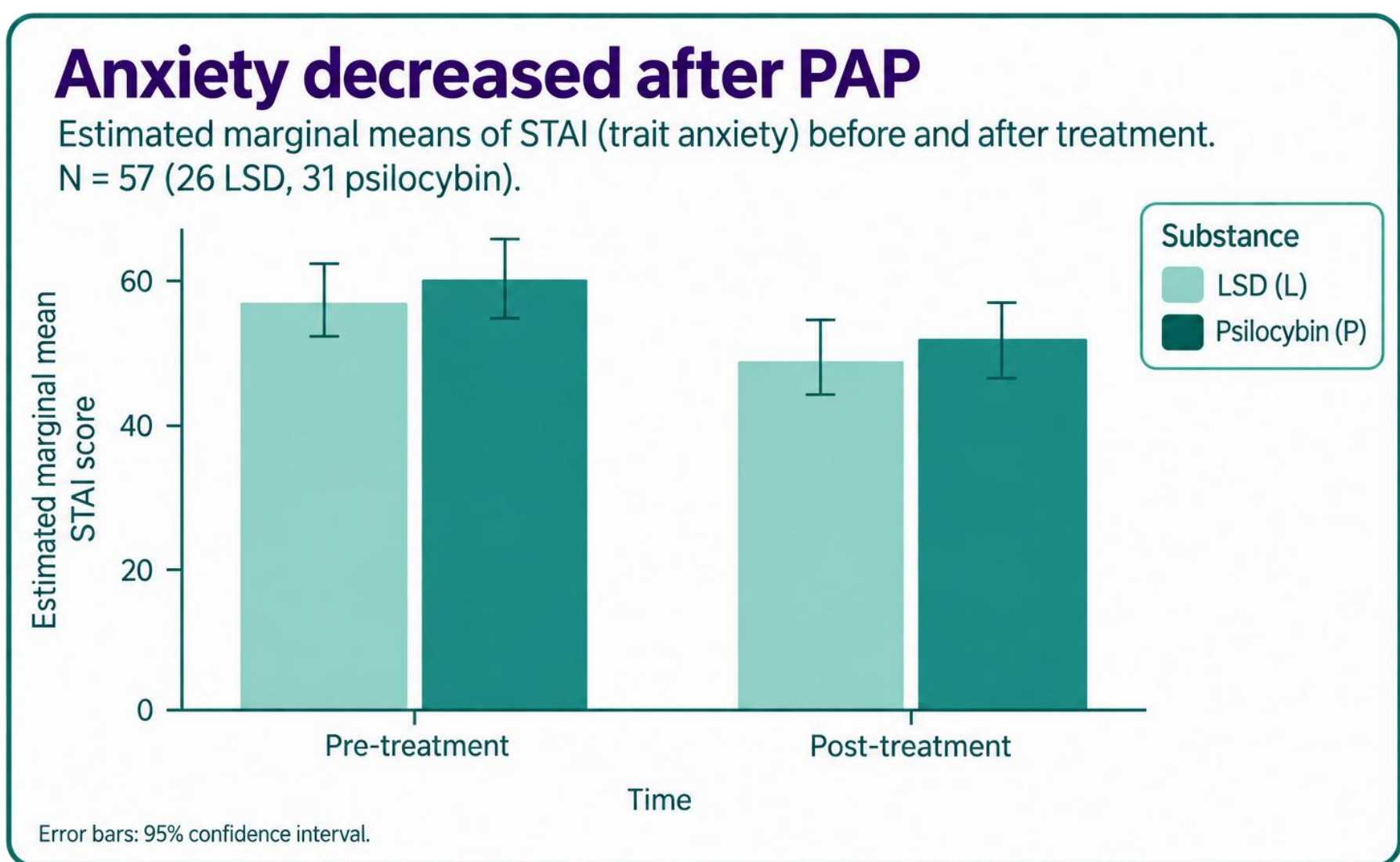
#### Secondary aims

- Assess **change in anxiety** before vs. after PAP.
- Compare anxiety outcomes **LSD vs psilocybin**.
- Explore whether mystical-experience **intensity varies with dose**.

### Methods



### Results



- Anxiety significantly decreased** after PAP across the 57 participants ( $F(1,55) = 23.2$ ,  $p < 0.01$ ), regardless of the psychedelic administered.
- No significant difference between LSD and psilocybin** was observed in anxiety outcomes ( $F(1,55) = 0.18$ ).
- Mystical-experience intensity was not significantly associated with anxiety improvement**: MEQ30 scores showed neither a significant interaction with STAI-T reduction.
- 48%** of participants met the criterion for **a complete mystical experience**.
- No clear dose-dependent relationship** with complete mystical experience was observed.

### Reflection and limits

The absence of a linear relationship between mystical-experience intensity and clinical improvement contrasts with a prominent hypothesis in the psychedelic literature. However, it does not necessarily exclude a therapeutic role for these experiences. Instead, it might raise questions about the measurement tool itself, its limitations, and, more broadly, how psychedelic science conceptualizes and assesses mystical experiences. This invites further reflection on how such experiences are defined and quantified in psychedelic research.

#### Limitation in the study

- Small sample size (N = 57)** and substantial **clinical heterogeneity**, including differences in comorbidities and functional impairment.
- Retrospective, non-randomized clinical design**, limiting standardization and causal interpretation.
- Variable treatment exposure**: psychedelic dose and number of sessions differed according to individual clinical needs.
- Selection bias**: compassionate-use patients had generally failed conventional treatments and represented a more severely affected, self-selected clinical population.
- Set and setting may have influenced phenomenology**: the neutral therapeutic environment may have favoured introspective experiences rather than explicitly mystical themes.

#### Some limits of the MEQ30

- Cultural and religious framing**: The vocabulary used carries historical and cultural assumptions. Although Stace's framework proposes an *ultimate unity* underlying mystical experiences across religious traditions, the concepts operationalized by the MEQ30 may still reflect a predominantly Euro-American and Christian-influenced understanding of mysticism.
- The limits of words**: Predefined categories and self-report statements may constrain how subjective experiences are described. Differences in educational, cultural, and spiritual backgrounds may also lead participants to interpret abstract or spiritual concepts differently.
- Oversimplification**: The threshold for a “complete mystical experience” may obscure partial or qualitatively different mystical-type experiences. Reducing a multidimensional experience to a global category or score may therefore limit more fine-grained analysis of its phenomenology.

#### What about other methods ?

- Phenomenological approaches**: Micro-phenomenology, for example, uses guided interviews to reconstruct fine-grained structures and temporal dynamics of lived experience.
- Semantic and computational approaches**: Free reports can subsequently be analysed using semantic similarity, topic modelling or other natural-language processing methods.
- Beyond the “mystical” label**: characterizing more specific dimensions such as awe, connectedness, ego dissolution, altered self-processing or oceanic boundlessness, rather than subsuming them all under a single mystical-experience construct.

#### Relevant references

- Saville-Smith R, Mosurinjoh S. The mystic trip: how psychedelic research fell for mysticism. *Psychopharmacology* (Berl). 2026 Jul 17. doi:10.1007/s00213-026-07119-3 PubMed PMID: 42467097.
- Petitmengin C, Remillieux A, Valenzuela-Moguilansky C. Discovering the structures of lived experience. *Phenom Cogn Sci*. 2019 Sep 1;18(4):691–730. doi:10.1007/s11097-018-9597-4 PubMed Central PMCID: PMC10171290.
- Maclean KA, Leoutsakos JMS, Johnson MW, Griffiths RR. Factor Analysis of the Mystical Experience Questionnaire: A Study of Experiences Occasioned by the Hallucinogen Psilocybin. *J Sci Study Relig*. 2012 Dec;51(4):721–37. doi:10.1111/j.1468-5906.2012.01685.x PubMed PMID: 23316089; PubMed Central PMCID: PMC3539773.
- Ko K, Knight G, Rucker J, Cleare AJ. Psychedelics, Mystical Experience, and Therapeutic Efficacy: A Systematic Review. *Front Psychiatry*. 2023 Apr 5;14:1077311. doi:10.3389/fpsy.2023.1077311 PubMed PMID: 36923458; PubMed Central PMCID: PMC10340484.
- Jones R, Gellman J. Mysticism [Internet]. 2004 Nov 11 [cited 2026 Sep 25]. Available from: <https://plato.stanford.edu/entries/mysticism/>
- Barrett FS, Johnson MW, Griffiths RR. Validation of the revised Mystical Experience Questionnaire in experimental sessions with psilocybin. *J Psychopharmacol*. 2015 Nov;29(11):1182–90. doi:10.1177/0269881115609019 PubMed PMID: 26442957; PubMed Central PMCID: PMC5203697.